

Read the stage, then look at the macula

Neovascularisation defines proliferative disease; macular damage is a separate assessment.

Retinopathy stage

Stage	Defining feature
Mild NPDR	Microaneurysms without more advanced criteria.
Severe NPDR	4 quadrants with >20 haemorrhages each; or venous beading in at least 2 quadrants; or IRMA in at least 1 quadrant.
PDR	New vessels on the disc or elsewhere.
Advanced disease	Persistent blood, tractional detachment or neovascular glaucoma.

Clinical meaning

Disease can be asymptomatic despite significant retinal changes. Cotton wool spots reflect nerve fibre infarction; hard exudates reflect lipid and protein leakage. Stage the retinopathy, then examine the macula separately. Useful central acuity does not exclude important peripheral disease or the need for surveillance.

References Ashley card 1700494472710; Elijah para 4536; Elijah para 4524; Ashley card 1701958668730; Elijah para 4537; Elijah para 4538; Elijah para 4539

Macular oedema and proliferative disease need different targets

Central fluid can threaten vision even when the retinal disease is non-proliferative.

Different treatment targets

Which component threatens vision?

Macular fluid

OCT assesses thickening and fluid; anti-VEGF can treat oedema.

New vessels

PRP reduces peripheral oxygen demand and VEGF drive.

Systemic risks

Improve diabetes, blood pressure and lipid control.

Clinical meaning

Macular oedema can occur with any NPDR or PDR stage. Enlarged foveal avascular zone on angiography suggests macular ischaemia. PRP generally targets retina outside the temporal arcades, rather than the macular centre. Name retinal stage and macular involvement independently when reporting the examination.